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VARIATIONS GUIDELINE FOR HUMAN AND VETERINARY MEDICINES

This document provides recommendations for applicants submitting variation applications for human and veterinary medicines. In addition to this guideline, the South African Health Products Regulatory Authority (SAHPRA) reserves the right to request additional information to establish the safety, quality and efficacy of a medicine based on the knowledge available at the time of evaluation. SAHPRA is committed to ensuring that all registered medicines meet the required quality, safety and efficacy.

Document History

Final Version	Reason for Amendment	Effective Date [dd Month yyyy]
1	Publication for comment	[10 May 2019]
2	Adds alterations to codes B.IV.1.a.1 (changes procedure type to Type IB) and B.IV.a.2 (eliminates restriction of the code to be for veterinary products only); Excludes code A.2.a based on non-applicability.	[15 November 2019]
3	Modifies addendum items 4.1.3 and 4.1.7 to include the addition of contraindication as a Type IB. Amends the documentation requirements in tables 5.2.1 & 5.2.2. Introduces the following quality sections to allow SAHPRA an additional review period prior to implementation (4.3.1 - 4.3.4; 4.3.7; 4.3.10 - 4.3.12; 4.3.15) Introduces additional document requirements in 5.4. Provides the requirement of a resolution letter in sections 4.3.8 and 4.3.9	[01 June 2020]
4	Update to remove additional review period and include alignment of certain quality codes to EMA classification. Alignment with EMA guidelines on Type IA and IA _{IN} in terms of SAHPRA response timelines from date of notification. Update includes revisit of Inspectorate timelines and commitment. Inclusion of documentation for submission of quality and administrative changes are defined. Inclusion of administrative change in proprietary name on certificate. Update on interpretation of quality variation fees. Update of templates regarding SAHPRA address change and removal of CD/USB submissions.	[29 September 2020]
5	Inclusion of variation timelines in section 3. Modifies addendum item 4.1.14 to include procedural exceptions regarding Type IA _{IN} veterinary applications.	[01 February 2022]

	Introduces document requirements in section 5.2. Update on interpretation of variation fees. Amendment of calendar days to working days.	
6	More details added to 4.4.2. Included Types of variations under 5.1. General Included the email address for PEM Quality variation application related queries under 5.3. Quality Additional information included for Points 3, 11, 12 & 14 in Table 5.3.1	[30 September 2022]
7	Consolidation of the variation communication and Variations Addendum guideline. Addition of new variation codes. Adaptation of EMA variation classifications and integration of SAHPRA specific requirements. New cover letter template for post-reg variations - [GLF-HPA-06G]	[09 August 2026]
8	Watermark update Document number updates of the new variation codes. Application letter template update to include PV/Clinical N/S alignment of review working days Update of Clinical code C.I.10 Minor alignment updates to Quality and Certification Codes	[09 August 2026]

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Glossary

Abbreviations/ Descriptions	Meaning
Amendments	Used interchangeably with the term ‘variations’
APIMF	Active Pharmaceutical Ingredient Master File Equivalent to ASMF: Active Substance Master File (EU terminology)
BP	British Pharmacopoeia
BSE	Bovine Spongiform Encephalopathy
BTIF	Bioequivalence Trial Information Form (OF-PEM-PRE-01J)
CCDS	Company Core Data Sheet
Clone	Application submitted by the innovator as a copy of its own product under a different proprietary name at any stage during the product life cycle.
CMC	Chemistry Manufacturing Controls
CP	Comparability Protocol
CTD	Common Technical Document Equivalent to: EU-CTD (EU terminology)
COVID - 19	Corona Virus Diseases 2019
DDPS	Detailed Description of the Pharmacovigilance System
DHCP	Dear Healthcare Professional (letter)
DNA	Deoxyribonucleic Acid
DVP	Dear Veterinarian, Professional (letter)
EC	European Commission
eCTD	Electronic Common Technical Document
EMA	European Medicines Agency

EU	European Union
HCR	Holder of the Certificate of Registration Equivalent to MAH: Market Authorisation Holder (EU terminology)
HPLC	High-Performance Liquid Chromatography
ICH	International Council for Harmonisation of Technical Requirements for Pharmaceuticals for Human Use
ICSR	Individual Case Safety Report
Label	Equivalent to: Labelling (EU terminology)
Legal Status	Includes, but is not limited to, Scheduling Status in South Africa
MAH: Market Authorisation Holder	Equivalent to HCR: Holder of the Certificate of Registration
MCC	Medicines Control Council (now SAHPRA)
MCB	Master Cell Bank
Medicine	Equivalent to Medicinal Product (EU terminology) ^{1N&S}
MSL	Master Seed Lot
N&S	Names and Scheduling
PASS	Post-authorisation Safety Studies
PBRER	Periodic Benefit-Risk Evaluation Report
PEM	Pharmaceutical Evaluation Management (PEM)
PI	Professional Information Equivalent to SmPC: Summary of Product Characteristics (EU terminology) ¹
PIL	Patient Information Leaflet Equivalent to Package Leaflet (EU terminology)
PMF	Plasma Master File

PSUR	Periodic Safety Update Report
PV	Pharmacovigilance
QA	Quality Assurance
QC	Quality Control
QIS	Quality Information Summary
QOS	Quality Overall Summary
QP	Qualified person; where reference is made to the qualified person, if the manufacturer is in South Africa, this refers to the Responsible Pharmacist. Where manufacture is conducted outside South Africa, the qualified person as defined by EMA may be applied.
QPPV	Qualified person responsible for pharmacovigilance
RMP	Risk Management Plan
RP	Responsible Pharmacist
RRA	Recognised Regulatory Authority – a term used to refer to the list of regulatory authorities with which SAHPRA aligns itself.
RSA	Republic of South Africa
SOP	Standard Operating Procedure
TOA	Transfer of Applicancy
The Authority	Relevant regulatory authority, in this case SAHPRA
ToHCR	Transfer of the Holder of Certificate of Registration
TSE	Transmissible Spongiform Encephalopathy
USRN	Urgent Safety Restriction Notice
USP	United States Pharmacopeia
VAMF	Vaccine Antigen Master File

VICH	International Cooperation on Harmonisation of Technical Requirements for Registration of Veterinary Medicinal Products
WCB	Working Cell Bank
WFI	Water for Injection
WHO	World Health Organisation
WSL	Working Seed Lot

Definitions of Terms

TERM	MEANING
Adjuvant	A substance or combination of substances used in conjunction with a biological medicine to achieve (for example, increase, accelerate, prolong and/or target) or modulate a specific physiological or immune response to the biological in order to enhance the clinical effectiveness of the biological medicine.

TERM	MEANING
Antigen (vaccine)	The following definitions apply in this document: • The active ingredient in a vaccine against which the immune response is raised. It may be a live attenuated preparation of bacteria, viruses or parasites; inactivated (killed) whole organisms; crude cellular fractions or purified active biological ingredients, including recombinant proteins (i.e., those derived from recombinant DNA expressed in a host cell); polysaccharides and conjugates formed by covalent linkage of polysaccharides to components such as mutated or inactivated proteins and/or toxoids; synthetic active biological ingredients; polynucleotides (such as plasmid DNA vaccines); or living vectored cells expressing specific heterologous immunogens. It may also be a combination of the antigens or immunogens listed above. • Intermediate or component that may undergo chemical change or processing in the manufacture of the final product (drug product) and is present in the final product in a modified form intended to furnish the specified activity or effect. Also referred to as Drug Substance, Active Ingredient, or Active Substance in other documents.
Biological Medicine	All medicines that contain a living organism or are derived from a living organism or biological processes. They include, but are not limited to the following: - Plasma-derived and animal products e.g. Clotting factors, immunosera, antivenoms - Vaccines - Biotechnology-derived medicines (recombinant DNA products), for e.g., rHu-anti-haemophilic factors, hormones, cytokines, enzymes, monoclonal antibodies, erythropoietins, nucleic acids - Products developed for Human Gene therapy. Well-characterised, low-molecular mass, medicinal biological compounds, may be excluded by specific regulatory decisions from the biological medicine status,

TERM	MEANING
	and in that case, will not be reviewed through the biological medicines review process.
Change	Refers to any change made to an approved marketing authorisation in product composition, active and non-active ingredients, manufacturing process, quality controls, equipment, containers, shelf life, storage conditions, facilities, or product labelling information by the marketing authorisation holder. This is also referred to as a variation in other documents.
Certificate Variation	Transfer of Applicant and Proprietary Name Change Variation
Comparability Exercise	The activities, including study design, conduct of studies, and evaluation of data, which are designed to investigate whether the products are comparable. In addition to routine analyses performed during production and control of the active biological ingredient or final product, these evaluations typically include further characterisation studies. In some cases, non-clinical or clinical data might contribute to the conclusion.
Comparability Protocol (CP)	The CP establishes the tests to be done and acceptable limits to be achieved to demonstrate comparability of the pre-amendment and post-amendment products following specific quality change(s). A CP is a highly specific, a well-defined plan for the future implementation of a quality (e.g., manufacturing-related changes, change of analytical method and site transfer change. It is also referred to as Post Approval Change Management Protocol in other documents.
Container Closure System	This system refers to the following components: • A primary container closure system is a packaging component that is or may be in direct contact with the final product dosage form (e.g., vial, pre-filled syringe). <i>(Container closure systems for the active biological ingredients or intermediates of medicines normally only include primary container closure systems.)</i> • A secondary container closure system is a packaging component that is not and will not be in direct contact with the dosage form (e.g. carton, tray).
Dosage Form	This is the pharmaceutical form in which the active ingredients, excipients and physical formulation of a medicine is presented.

TERM	MEANING
Final Lot	A collection of sealed final containers that are homogeneous with respect to the composition of the product. A final lot must therefore have been filled in one continuous working session.
Final Product	A finished dosage form (e.g. tablet or solution) that contains an active ingredient, but not necessarily, in association with inactive ingredients. It is also referred to as a Finished Product or Drug Product in other documents.
Formulated bulk	An intermediate in the drug product manufacturing process, consisting of the final formulation of drug substance and excipients at the concentration to be filled into primary containers.
Holder of the Certificate of Registration (HCR)	An HCR refers to a person or legal entity in whose name a registration certificate has been granted and who is responsible for all aspects of the medicine, including quality, safety, and compliance with conditions of registration.
Intermediate	A material produced during steps in the manufacturing of a medicine that must undergo further processing before it becomes a final product.
Manufacturer	This refers to a person manufacturing a medicine and includes a manufacturing pharmacy.
Master cell bank (MCB)	An MCB is an aliquot of a single pool of cells, which generally has been prepared from the selected cell clone under defined conditions, dispensed into multiple containers and stored under defined conditions.
Prior approval amendment (PAA)	A PAA is an amendment requiring approval from SAHPRA prior to implementation of the amendment. Also referred to as change application dossier or Type II variation in other documents.

TERM	MEANING
Product Labelling Information	Printed materials that accompany a prescription medicine and refer to all labelling items as per the Medicines and Related Substances Act, 101 of 1965, General Regulations # 10, #11 and # 12): • Professional Information (PI), including prescribing information, which provides product information on indication, dosage and administration, safety and efficacy results, contraindications, warnings, and a description of the product for health care providers. • Inner label or container label • Outer label or carton • Patient Information Leaflet (PIL)
Quality attribute	A physical, chemical, biological or microbiological property or characteristic.
Quality change	In the context of this document, quality change refers to a change in the manufacturing process, product composition, quality control testing, equipment, or facility.
Raw materials	A general term used to denote the culture media components, reagents or solvents intended for use in the production of starting material, drug substance, intermediates, or drug products.
Registration Application	A formal application to SAHPRA for approval to register and market a new medicine. The purpose of the Registration Application is to determine whether the medicine meets the statutory standards for safety, effectiveness, product labelling information and manufacturing.
Source material/starting material	Material from a biological source that marks the beginning of the manufacturing process of a drug as described in a marketing authorisation or licence application and from which the active ingredient is derived either directly (e.g., plasma derivatives, ascitic fluid, bovine lung, etc.) or indirectly (e.g., cell substrates, host/vector production cells, eggs, viral strains, etc.).
Specification	A list of tests, references to analytical procedures, and appropriate acceptance criteria which are numerical limits, ranges or other criteria for the tests described. Specifications are critical quality standards that are proposed and justified by the manufacturer and approved by the regulatory authorities.

TERM	MEANING
Vaccine	Preparations containing antigens capable of inducing a specific and active immunity in humans against an infectious agent or toxin.
Vaccine Efficacy	Relative reduction in disease incidence in vaccinated people compared to unvaccinated people measured in a randomised, placebo-controlled clinical trial. In the context of this guidance document, vaccine efficacy relates to all clinical data obtained to ensure vaccine efficacy, immunogenicity, or its effectiveness.
Working Cell Bank (WCB)	The WCB is prepared from aliquots of a homogeneous suspension of cells obtained from culturing the master cell bank under defined culture conditions.

APPROVED

1. INTRODUCTION

The Variations Guideline includes information on the various categories and codes for variations and documents that applicants are required to submit.

The submissions will be evaluated as indicated in the Medicines and Related Substances Act.

1.1 Purpose

The purpose of this document is to inform applicants on how to submit applications for variations to the South African Health Products Regulatory Authority (SAHPRA) for Category A and C medicines as well as what to expect during assessment of variations.

1.2 Scope

The South African Health Products Regulatory Authority (SAHPRA) has compiled a list of requirements for submission of variations to registered products for human and veterinary medicines as well as biologicals. We encourage applicants to read this guideline together with the relevant classification and codes (Section 3.4 below) and SAHPRA guidelines relevant to each Technical Unit as well as the recent relevant guidelines and templates listed in Appendix A.

We have outlined requirements in this guideline as well as classification and codes that are aligned to the framework of the International Council for Harmonisation: Technical Requirements for Pharmaceuticals for Human Use (ICH) and the International Cooperation on Harmonisation: Technical Requirements for Registration of Veterinary Medicinal Products (VICH). SAHPRA commits to global best practices regarding the safety, quality and efficacy of health products.

2. LEGAL PROVISION

The Medicines and Related Substances Act No. 101 of 1965, as amended, Section 15. (3) (a) (i) - (iii), specifies medicines, medical devices and/or In Vitro Diagnostics (IVDs) that require registration, which should comply with the prescribed requirements and be safe, efficacious and of good quality. General Regulations of the Medicines and Related Substances Act, Regulation 53 (1) and (2) requires that changes to accepted standards or specifications of the registered product should be submitted to SAHPRA for prior approval and the latter should be granted before the changes are implemented.

3. CLASSIFICATION OF VARIATIONS

3.1 Type IA variations (Notifications):

Minor changes that have minimal or no impact on quality, safety, or efficacy of the product.

Type IA variations are split into two categories, with differing procedural treatment:

- a) **Type IA_{AN}:** Minor variations that do not require any prior approval but must be notified by the HCR within 12 months following implementation ('Do and Tell' procedure).
- b) **Type IA_{IN}:** Minor variations that must be notified by the HCR at least 47 working days prior to implementation. If 47 working days lapse from the time the notification was submitted, and SAHPRA has not provided feedback to the HCR, the HCR may implement the variation.

Upon submission of notification of Type IA_{IN} to SAHPRA, the Authority may review the Type IA_{IN} notification within 47 working days.

If SAHPRA has not sent the HCR its opinion on the application within 47 working days, the application will be deemed implementable. Applicants must satisfy themselves that they meet all the prescribed conditions for the change and submit all required documentation with the submission of variations. The variation codes that require approval from more than one (1) Technical Unit cannot be implemented until approval is received from the respective Technical Unit.

Note: Failure to meet the specified conditions in the case of notifications of Type IA and minor variations of Type IA_{IN} or provide all necessary documentation in the application, may lead to the immediate rejection of the variation, even if the review time has elapsed. If the rejection letter specifies that the implementation of change should be reversed, and the HCR should withdraw the impacted sections of the dossier and revert to the previously approved version within 15 days.

If any of the conditions are not met or documents are not available, the variation should be classified as a Type IB and submitted as such.

3.2 Type IB variations (minor variations):

Type IB variations are changes that may have minor impact on the overall safety, efficacy and quality of the health product or medicine. Applicants must submit all required documents when they submit application for the variation.

SAHPRA may review a minor variations of Type IB submission within 87 working days, unless otherwise indicated. If SAHPRA has not sent the HCR its opinion on the application within 87 working days, the application will be deemed implementable (as applicable depending on the variation code).

Failure to meet the specified conditions in the case of notifications of minor variations of Type IB or provide all necessary documentation in the application, may lead to the immediate rejection of the variation, even if the review time has elapsed.

The timeline for evaluation of Type IB variations is 87 working days. If no response has been received from SAHPRA within this time, changes may be implemented, however SAHPRA may still reject, request reversal of implementation or request further data after the time has elapsed. Please note that SAHPRA timelines are calculated in working days. If the rejection letter specifies that the change should be unimplemented, the HCR should revert the impacted sections of the dossier back to the latest approved version by means of a closing sequence within 15 days.

Applications which cannot be implemented after 87 working days include changes which need approval from more than one (1) Technical Unit, e.g. change in manufacturer of the finished product, change in primary packer or changes in the PI/PIL.

3.3 Type II variations:

Type II variations are changes that could have major impact on the overall safety, efficacy and quality of the health products and/or medicines. The submission for review must be approved by SAHPRA before implementations and the documents required for the changes must be submitted together with the submission.

SAHPRA has included the Type II variations classification in this guideline (refer to the relevant classification and codes as referenced below) with a view to facilitating timeous review and approval of Type II variations.

Upon submission of Type II variations to SAHPRA, the Authority will review the Type II submission within 220 working days for all variations.

NOTE: This guideline lists only the minimum technical data requirements for Type II variations. Applicants are advised to include additional data to support the Type II variation if available. Please note that SAHPRA reserves the right to request further data if it is necessary.

Exceptions

The timeline for Certificate Variations applications is 30 working days.

3.4 Classification Codes:

For classifications and codes for each of the above-mentioned types of variations, please refer to the following classification and codes under respective Technical Unit on <https://www.sahpra.org.za/guidelines/>:

- a. GLF-HPA-06C: Clinical (CEM)
- b. GLF-HPA-06D: Names and Scheduling (N&S)
- c. GLF-HPA-06E: Inspectorate (INS)
- d. GLF-HPA-06F: Biological medicines (PEM)
- e. GLF-HPA-06H: Certificate Variations (HPA)
- f. GLF-HPA-06I: Quality (PEM)
- g. GLF-HPA-06J: Veterinary

3.5 Where to submit applications

The SAHPRA Engagement Portal is an online platform designed to facilitate interactions between applicants and SAHPRA. The portal enables applicants to submit and track applications, manage their profiles and ensure compliance with regulatory requirements. File Transfer Protocol (FTP) details must be set under the Organisation Details before attempting to create applications that require eCTD uploads to the SAHPRA FTP Server. A request for the FTP User Guide for Files/Dossier Submission and access to the FTP should be sent to variationsuploadqueries@sahpra.org.za. When requesting access to the FTP, applicants should include the company name and licence number.

3.6 Submitting variations for duplicates and different strengths

In the case of identical variations being submitted for duplicate products or different strengths of the same health products and/or medicines, SAHPRA encourages the applicant to submit a combined dossier rather

than submitting multiple/separate dossiers. This reduces the workload burden on SAHPRA and facilitate faster overall turnaround time and ensures document completeness.

Refer to SAHPRA eCTD Specification 3.1 for additional information.

For submissions of PI/PIL variation applications where there are different strengths of the same product on one PI/PIL, only one application should be submitted. Separate PI/PIL must be submitted if they are in different dossiers

Note: The PI/PIL may need to be separated for variations for different strengths or schedules on the advice of the regulator.

For Clinical variation applications of masters/duplicates and clones the applicant should ensure that the proposed **clean** PIs and PILs have the **specific product** name(s) throughout the document (PI/PIL) instead of using 'PRODUCT NAME'.

Note: All clean proposed PI and PIL must be dated, initialled, and paginated as page X of Y.

3.7 Submitting grouped variations

3.7.1 Submission of multiple variations in a single application (grouped variation)

A grouped variation refers to a single application submitted for multiple changes to SAHPRA for a medicinal product. These changes can involve different types of variations, but they must be linked to the same marketing authorization and can be grouped together under specific conditions. While grouping of different types of variations (Type IA, Type IB and Type II) is acceptable, variations for different Technical Units should not be grouped in a single submission. For information on the acceptable number of variations to be grouped in a single submission refer to SAHPRA engagement portal communications.

There should be a clear link or relationship between the grouped variations. For example, a Type II variation for a new indication might be grouped with Type IA in variations for PI/PIL updates affecting the new indication.

Type I variations may be submitted with Type II variations; however, the Type I variations are not implementable until the entire variation submission is approved. Applicants found to have implemented Type I variations prior to the approval of Type II variations within the same submission will have all variations related to the specific submission rejected.

For fees applicable to grouped variations refer to Regulations regarding Fees Payable in terms of the provisions of the Medicines and Related Substances Act related to quality.

For clinical variations, each variation in the grouped submission will be charged as per classification code in accordance with the type of variation submitted. Each variation must be clearly identified as outlined in GLF-HPA-06C: Clinical (CEM) and will be charged individually based on its classification. For PV recommendations added to the PI, each recommendation must be accompanied by the corresponding classification code and payment. A fee will be charged per PV recommendation submitted.

In general, it is recommended that all identified changes related to the product dossier must be submitted as individual variations in separate submissions with the applicable fees. There should be no grouping of variations related to the same module in the dossier, unless the changes are related. The exception to this is PI/PIL amendments and the annual notifications (Type IA only) for changes made to the dossier in compliance with the pharmaceutical quality system of the manufacturer.

Applicants are advised NOT to combine either Proprietary Name Change or Transfer of HCR variation applications with any Type IA, Type IB or Type II variations in the same application, to streamline the evaluation process. Applicants are advised to finalise the Transfer of HCR variation (approval from Inspectorate and Issuance of the Registration Certificate) before they can apply for the Proprietary Name Change variation. Applicants must submit the registration certificate to the Clinical or Biological Unit when applying for variations related to the Proprietary Name Change or Transfer of HCR in the PI/PIL.

3.7.2 Submitting Quality and Clinical variations together

Quality and Clinical variations, that require review by both units, should be submitted. The application will be routed to the relevant unit and will be evaluated sequentially separately. The relevant Quality and Clinical variation fees will be applicable.

For clinical variations, where amendments have been made on the Quality section of the PI/PIL, the applicant is required to submit the relevant supporting documents from the quality unit. For Type IA *notification, proof of submission will be required. For Type IB and Type II variation, the corresponding Quality Unit approval letter must be submitted.* Applicants to note that Amendments to any quality aspects of the medicine within the PI/PIL are subject to Quality Unit approval and may only be implemented in the PI/PIL if approved by the Quality unit. Where the applicant is just bringing the PI/PIL in line with current PI/PIL guidelines and the Quality aspects were approved at registration the applicant may submit a declaration to confirm that the Quality aspects were approved at registration, and no unsolicited changes have been made.

NOTE FOR BIOLOGICAL MEDICINES: Quality variations that affect the PI & PIL. The applicant is required to provide the annotated PI & PIL as part of the supportive documents with the quality change reflected (e.g shelf-life extensions, container closure and excipient changes). This change will not be handled separately.

3.7.3 Submitting Quality and Inspectorate variations together

Quality and Inspectorate variations that require review by both units must be submitted separately with the applicable fee but in parallel.

Replacement or addition of a manufacturing site for part or all of the manufacturing process of the finished product and/or addition/replacement of a primary packaging site of the finished product will require evaluation by Quality unit as well as Inspectorate unit.

The respective fees applicable for both the Quality and Inspectorate variations must accompany the submissions.

The replacement or addition of a finished product site does not require an update to the registration certificate. The variations summary will serve as an addendum to the registration certificate.

NOTE: This change is not implementable unless approval is received from both the Quality and Inspectorate Units

3.7.4 Submitting a Transfer of Applicancy and Verification of GMP compliance for a Proprietary Name Change to Inspectorate.

For a Transfer of Applicancy (TOA) where there is a subsequent Proprietary Name Change variation, the applicant is required to first submit the TOA together with the verification of GMP compliance for Proprietary Name Change to the Inspectorate Unit using the relevant Inspectorate variation code. Once approval is

granted by the Inspectorate Unit, the applicant must submit the Proprietary Name Change variation to the Naming and Scheduling Unit within 30 working days.

3.8 Documentation for reliance

SAHPRA has developed reliance pathways to streamline the application approval process. To facilitate this process, Applicants are reminded of the reliance requirements below.

- In order to encourage the maintenance of reliance throughout the lifecycle of the product, different Technical Units in SAHPRA have established reliance mechanisms. Refer to the specific sections of the respective Technical Units in this guideline or the relevant classification and codes for details on applying reliance to registered products.
- Full assessment/evaluation reports are required should the application have received approval from Recognised Regulatory Authorities (RRAs).
- SAHPRA will be implementing reliance models for qualifying applications. The General Information Guideline (SAHPGL-HPA-07) and Reliance Guideline (SAHGL-BAU-01) contain the latest information regarding SAHPRA's evaluation pathways as well as SAHPRA's Recognised Regulatory Authorities (RRAs) and collaborative / work sharing procedures.
- To apply for a verified review, applicants are encouraged to leverage SAHPRA's repository of PI/PILs. Additional information is provided in the Clinical Guideline (SAHPGL-CEM-01), and Quality and Bioequivalence Guideline (SAHPGL-PEM-02).

3.9 Fees

The fees applicable to variations are published in the Government Gazette, Regulations regarding Fees Payable in terms of the provisions of the Medicines and Related Substances Act.

Refer to SAHPGL-FIN-01 SAHPRA Payment Guideline for the details of the payment of fees to SAHPRA.

The Proof of Payment and Portal Sales Order should be included in module 1.2.2.1.

Please note that the fees payable for respective Technical Units are independently charged as these variations are reviewed independently.

For veterinary medicines, the fees for quality variations are under discussion.

3.10 Submission requirements for priority review applications (TYPE II applications only)

Kindly refer to the current version of the communication regarding priority review requests published on the SAHPRA website.

3.11 Applying for a withdrawal of a variation submission

If a withdrawal of a variation submission is necessary, submit the withdrawal of the variation in a follow-up sequence via the SAHPRA Engagement Portal in line with the current eCTD specifications.

The following relevant supporting documents are required:

- The letter includes the motivation for the withdrawal of the variation submission, signed by the RP or

- authorized pharmacist responsible for signing the dossier.
- Proof of submission of the variation application.

3.12 Submitting the QIS/QOS

The QIS (Quality Information Summary) document is a product lifecycle management document, and it is important that it is updated with each variation made to the product, which affects the QIS.

It is required that an updated and complete QIS be submitted with all quality variation submissions, if affected by the change. Only the sections that have been amended should be highlighted in yellow and the sections that have been replaced should have a strike through for e.g. ~~(strike)~~. Once the variation is approved, the applicant can replace the struck through information on the QIS document with the newly approved data. A closing sequence may be submitted to SAHPRA to replace the annotated QIS document with the approved clean QIS document.

A QOS (Quality Overall Summary) is only required for Type II API source change variations. Only the sections of the QOS pertaining to the new source (3.2.S) should be completed.

Applications requiring a QIS document but submitted without, will be rejected and the applicant will be requested to resubmit the application according to the requirements.

Note: The QIS/QOS is not a requirement for Certification, Clinical, Veterinary and Biological Medicines (Quality and Clinical) Variations.

3.13 Submitting the variation validation template

The purpose of the Variation Validation Template is to verify that all the required information has been submitted to SAHPRA to evaluate the variation submission. It is also used for follow-up sequences that may be required for the variation.

Applications submitted without the Variation Validation Template will be rejected and the applicant will be requested to resubmit the submission according to the requirements.

Applicants must include a fully completed validation template in the applicable section for the submission. A PDF and word version of this document should be included in Module 1.2.5.

For z-code applications: all conditions and all documentation requirements are applicable as far as is relevant. Alternatively, the Applicant may populate Z-code information in Type II section of the validation template (Section C).

3.14 Format change to the PI and PIL

The format for Professional Information and Patient Information Leaflets is reflected in the updated SAHPRA SAHPGL-CEM-02 and SAHPGL-CEM-03 guidelines.

Note: PI changes to the EMA format are also applicable to veterinary medicines.

Repository of PIs and PILs

SAHPRA has published a repository of PIs and PILs on its website for the benefit of health care providers and patients, as well as to enable streamlined Clinical evaluations of applications for generic medicines. Where available for a given molecule, applications for generic medicines are required to reference the latest

published SAHPRA-approved innovator PI in the application. Clinical screening queries will be immediately flagged for applications referencing an outdated/illegible PI where the latest version has been published on SAHPRA's website at the time of submission.

Note that the published PIs on SAHPRA's website may also be applicable to selected variation applications (e.g., safety update of a generic medicine where the same change has already been approved for the reference local innovator medicine).

The final dated versions of the generic and innovator PI/PIL for which an approval letter has been sent to the HCR/MAH, must be submitted via docubridge in a closing sequence using the Sequence Type "Closing sequence" and Sequence Description "Approved updated PI/PIL" for uploading onto the PI and PIL repository within 10 working days. Non-compliance may result in withdrawal of variation approval.

In addition, for Schedule 0-2 products, the applicant must upload the approved PI/PIL onto the SAHPRA OTC repository within 10 working days from receipt of the approval letter. Non-compliance will be flagged, and the applicant must rectify the omission within two (2) working days of notification. Withdrawal of the variation approval will be enforced if the omission is not corrected within this timeframe.

Note: For veterinary medicines all the PIs are attached to the vet medicines database as well.

For quality variations affecting the PI/PIL (e.g., change in appearance of FPP, change in container, storage conditions etc.), include a revised (annotated) PI/PIL in your submission (in Module 1.3).

3.15 De-linking of dossiers

For more information, please refer to the SAHPGL-HPA-09 SAHPRA ECTD SPECIFICATION 3.1 and Guidance for Module 1 and Regional Information.

3.16 Request for extension

An applicant may, prior to the expiry of the prescribed response period, submit a written request for an extension. No more than one extension shall be permitted, and any such extension shall not exceed 60 working days unless otherwise specified in the applicable processes of the relevant Technical Unit. A written request should be submitted, accompanied by a motivation letter within 30 working days from the date of the recommendation. Such motivation should be addressed to the following respective email addresses:

postregqualityvariations@sahpra.org.za

biologicals@sahpra.org.za

clinical.variations@sahpra.org.za

namesandscheduling.variations@sahpra.org.za

variations.inspectorate@sahpra.org.za

The motivation letter should include the product name(s), application number(s) and be submitted along with a copy of the recommendation letter.

Consult the fees guideline for appropriate fees for second response round. Submissions that do not address all queries by the second response round will be rejected.

Please note that if an applicant does not respond to the query within the specified timeframe and does not request for extension, the application will be rejected.

Applicants are reminded that new variations/changes may not be included in response submissions. These new variations should be applied for, in a separate submission. If new variations are of high priority, e.g., variations related to the implementation of pharmacovigilance recommendations, the response-variation application should have proof of payment for the additional variation attached.

The latest SAHPRA fees gazette should be consulted for the appropriate fees that should be paid.

3.17 Unforeseen changes (z-codes)

For PEM Post Registration and Biologicals, any unforeseen changes that have not been detailed in the classification and codes of this guideline should be submitted as a “z” code request.

Applicants are to submit “z” code variation requests, for unforeseen variations, to the relevant technical unit using the dedicated mailbox (postregqualityvariations@sahpra.org.za or biologicals@sahpra.org.za) using the z-code Request Form (**GLF-HPA-06A**). This form is accessible on the SAHPRA website and must accompany the z-code request email. The Applicant must ensure that the z-code request form and email are included in the submission. z-code requests are product and sequence specific and may not be re-used for other submissions. The relevant Technical Unit will inform the applicant within 20 working days of the correct classification, and the submission fee required via email. “z” Code submissions will not be accepted without prior approval from SAHPRA. The confirmation received from the technical unit should be included in Module 1.0.3, Correspondence from SAHPRA.

For variations submitted to PEM (Post Registration and Biologicals) please make sure that the documents as specified in Table 1 are included if applicable:

Table 1 – list of key documentation/data required for quality variation applications

1. Letter of application with (M1.0.1):
• Purpose of the variation(s)
• Description, Classification and Code of the Variation(s) (e.g. Type IB - B.II.b.3a)
• Where a variation involves other products, the applicant must indicate/highlight on the letter of application.
2. Application form (M1.2.1)
3. Proof of payment for the Variation application and fees breakdown for bulk payments (M1.2.2.1)
4. Letter of authorisation for communication on behalf of the applicant/PHCR (M1.2.2.2)
5. Dossier product information (M1.2.2.3) (If applicable)
6. API change control (M1.2.2.6) (If applicable)
7. The current approved PI and PIL (M1.3.1.1 & M1.3.2) (if applicable)
8. Amendment/variation schedule (M1.5.2.1)
9. GMP certificates (M1.7.3) (if applicable)
10. SAPC registration (M1.7.7)
11. Responsible pharmacist declaration (M1.7.7) when required.
12. Completed validation template for variation applications indicating whether conditions are met and supporting documents are included as per SAHPRA Variations Guideline Annexures (M1.2.5)
13. Batch Manufacturing Records (M3.2.R.7) for changes to the FP formulation, FP manufacturer, batch size, manufacturing process & equipment, in process control specification (if applicable)
14. Amended QIS (and QOS where applicable) document (M3.2.R.8) (If applicable)

15. <i>Reliance documentation – to be supplied for abridged and verified reviews</i>
16. Full assessment/evaluation reports from RRAs, if available (M1.10.5)
17. *The submitted specifications and analytical procedures must be signed, dated and version controlled. *For addition of API, the FPP manufacturer data must be submitted (3.2.S.4 & 3.2.S.5) * The QIS (and QOS where applicable) is a requirement, and this should not change the prescribed review timelines of 47 working days for Type IA, 87 for Type IB and 220 working days for Type II.

3.18 Clinical Post Registration

For variation application to other units affecting the PI and PIL:

Where the variation is a Type IA notification, proof of submission will be required.

Where the variation is a Type IB and/or Type II, an approval letter will be required.

3.18.1 Submission of Type I and II variation application(s)

For variation applications affecting the PI and PIL, approval from the Names & Scheduling Unit must first be obtained before submission of the variation application to the Clinical Unit. For variations affecting the PI and PIL from the PEM Unit, applicants must provide proof of submission.

The submission of Type I and II variation application(s) for evaluation should be classified according to SAHPRA CODES AND VARIATION CLASSIFICATIONS.

Types of submissions that are prohibited

1. Parallel submissions of variation application(s) are not acceptable i.e. 1. simultaneous submissions of separate variation application(s) of the same product, 2. submission of variation(s) whilst there is a variation application in progress.
2. In the event were the applicant self-implements changes to the PI/PIL (Type IA in or Type IB) due to the elapsed evaluation time and subsequently receives a non-approval letter from the Authority, the self-implemented PI/PIL cannot be used as the current approved PI/PIL. In this case the applicant will be required to retract and address the deficiencies.
3. Submission of a new or updated risk management plan (RMP) post-registration that does not affect the PI/PIL should be submitted in a follow-up sequence via SAHPRA Engagement Portal and should be directed to Pharmacovigilance Unit.

3.19 Period for response submissions and number of query rounds

For responses to technical screening queries, only 1 (one) query response round will be permitted. If the applicant fails to address the queries adequately, the application will be rejected, and the applicant shall submit a new application(s), and the applicable fee shall be payable.

For responses to evaluation recommendations/queries, only 2 (two) query response rounds will be permitted. If the applicant fails to address the queries adequately by the 2nd (second) query round, the

variation application will be rejected, and the applicant shall be required to re-submit the variation application, with the applicable fees.

3.20 Names and Scheduling

Variations applications are categorized as below:

- Proprietary name change: Type II - Code PNC-1-(H,V,CH,CV)
- Rescheduling application: Type II – Code RSC-2-(H,V,CH,CV)
- Authorized Prescribers Amendment: Type II – APA-3-NS

N.B. All Names and Scheduling Type II applications require regulatory approval prior to implementation.

3.21 Proprietary Name Change Variation Applications

A guidance on the criteria applied by the South African Health Products Regulatory Authority (SAHPRA) when evaluating the suitability of a proposed proprietary name for a medicine intended for human or veterinary use is stipulated in the SAHPGL-CEM-NS-03 Guideline for Proprietary Names for Medicines.

The proposed proprietary name should be included in all the relevant modules. If the applicant has proposed multiple names for the product, then all proposed proprietary names should appear in the variation application. However, if only one name is proposed, then only one name may appear on all modules.

It is recommended that three (03) proposed proprietary names should be submitted for evaluation. Where more than 03 proposed proprietary names are submitted per application, only the first 03 proposed proprietary names will be evaluated per submission. The timeframe for Type II proprietary name changes is 220 working days.

Refer to 3.7.4 Submitting a Transfer of Applicancy and Verification of GMP compliance for a Proprietary Name Change to inspectorate.

3.22 Rescheduling Variation Applications

A guide on the criteria for rescheduling of registered medicine or substances is stipulated in the SAHPGL-CEM-NS-02 Guideline Scheduling of Substances and Medicines and SAHPGL-CEM-NS-01 Guideline for the Rescheduling of substances and medicines.

Variation applications for the rescheduling of a registered medicine or substance (Type II) for evaluation by the Names and Scheduling unit, must be submitted via the SAHPRA Engagement Portal. Refer to the SAHPRA eCTD Specification 3.1 for more information.

For rescheduling of an existing registered product to a lower schedule which is part of an existing dossier, a delinking from the master dossier will be required.

In the event of a rescheduling application for a product moving to a lower schedule, a unique proprietary name and a separate new registration dossier is required.

The proprietary name change variation application for the de-linked medicine, should be submitted as the next follow-up sequence and the applicant should amend the eCTD envelope to remove the details of the master.

Scheduling of medicines as approved and gazetted by the Minister of Health

Rescheduling recommendations that SAHPRA has approved will be submitted as an amendment to the published Schedules, for consideration by the Minister of Health and publication in the *Government Gazette*.

Applicants are required to update their PI/PIL and all relevant sections of the dossier for the registered product to align with the new requirements as published and gazetted by the Minister of health.

For Rescheduling Type II variation applications made to SAHPRA, the timeline will be 220 working days.

3.23 Authorized Prescribers Amendment

A guidance on the criteria and policies applied by SAHPRA when evaluating applications for amendments to the Schedules with the objective of specifying substances to be made available for prescription by designated categories of health professionals, other than medical practitioners or dentists, as provided for by section 22A of the Medicines and Related Substances Act, 1965 (Act 101 of 1965) refer to SAHPGL-CEM-NS-04 Scheduling of Substances for Prescribing by Authorised Prescribers other than Medical Practitioners or Dentists.

Authorized Prescribers Amendment applications are requests made to SAHPRA and follow a 220 working day timeline.

3.24 Inspectorate

All queries related to Inspectorate variation applications must be submitted via the dedicated email address: variations.inspectorate@sahpra.org.za

For Proprietary Name Change the inspectorate should verify the GMP status of the manufacturing site listed in the medicines register prior to an updated registration certificate being issued. Therefore, applicants must apply for GMP Site verification to the inspectorate using an Inspectorate code simultaneously when applying for Proprietary name change. Applicants are reminded to include valid GMP certificates/Licence/Resolution letter when applying for GMP Site verification for proprietary name change.

For a combined Transfer of Applicancy and Proprietary name change variations refer to heading 3.7.4 Submitting Proprietary name change and Inspectorate together.

3.25 Certificate Variations

Please note: Throughout the communication, whenever the terms registration certificate(s) or certificate(s) appear and the term old medicine letter has not been included, the assumption should be that the same applies for old medicine letters as well. Hence, if the variation application submitted is for an unregistered product (old medicine), then the term old medicine letter is applicable, except in distinctly specific circumstances wherein the term old medicine letter cannot be practically applicable.

Certificate variations must be applied for within 30 working days after the approval letters are received from the relevant units (Naming and Scheduling and Inspectorate), for which the required changes are to be made on the revised registration certificate.

Note: On submission of transfer of Holder of Certificate of Registration/proprietary name change applications, there should be no pending variations that affect the information on the registration certificate.

Only selected variation applications require the issuing of an amended registration certificate:

- Changes in the proprietary name of a product.
- Transfer of the Holder of Certificate of Registration.
- Combined transfer of Applicancy and proprietary name change.

The approval letters issued by the relevant units for all other changes (i.e. amendment to dosage form, changes in the manufacturer, change in formulation), that are reflected on registration certificate must be appended to the current registration certificate (Module 1.5.2.2.2).

For transfer of applicant and proprietary name change variations, a separate closing sequence to issue the revised registration certificate must be submitted and must include the approval letter issued by the naming and scheduling unit and/or approval letter issued by Inspectorate.

Applicants must submit these applications separately, without any technical variations e.g. adding a new/additional manufacturer and/or a primary packer. This will allow for a streamlined process for certification.

For transfer of Holder of certificate of registration or proprietary name change variations, a variation is only considered approved once a revised old medicine approval letter/registration certificate is issued.

Certification requires 30 working days to finalise the revised registration certificate, barring no query(ies) on the application.

If applicable, queries will be communicated to the applicant. The applicant will be requested to submit the correct information in a follow-up sequence. The applicant should also provide proof of upload of the response to the certification mailbox: certificationvariations@sahpra.org.za.

The proprietary name change variation application for the de-linked medicine must be submitted as a follow-up sequence and the applicant should amend the eCTD envelope to remove the details of the master.

The final approved proprietary name must be included in all relevant modules when submitting the closing sequence and certificate variation request.

3.26 Documentation for Reliance

SAHPRA has developed reliance pathways to streamline the application approval process. To facilitate this process, Applicants are reminded of the reliance requirements below:

Inspectorate reserves the right to reject or approve the inspection outcome from the inspection conducted by an Authority in which a GMP MRA with SAHPRA exists. Refer to the Guideline SAHPGL-INSP-02_v8- Guideline-on-Good-Manufacturing-Practice-for-Medicines, Section 3.2.

Standard documentation for Inspectorate variation applications (Refer eCTD Validation and Technical screening for Variation).

3.27 Documentation/ data requirements

3.27.1 General

All variation submission should include an application form, available on the SAHPRA website, the cover letter and tabulated schedule of amendments (see appendix).

3.27.2 Clinical and Pharmacovigilance

Table 2 below covers all the *potential* documentation/data requirements for a given submission (to be read in conjunction with both the SAHPGL-CEM-01 Clinical Guideline and 2.24 Guidance for the Submission of the South Africa CTD / eCTD – General & Module 1). This represents the key requirements for variations applications – applicants may submit other relevant documentation not listed in Table 2 as needed. Note that the MRF4 form is no longer required.

This is followed by Table 3, which provides the requirements for each code applicable to Clinical and Pharmacovigilance.

Table 2 – list of key documentation/data required for clinical variation applications

Standard documentation for variation applications

1. Letter of application with a company letter head (M1.0.1):
 - Purpose of the variation(s)
 - Description, Classification and Code of the Variation(s) (e.g. Type II – C.I.4)
 - Where a variation leads to or is the consequence of other variations to the term of the same registered medicine, a description of the relation between these variations should be provided in the appropriate section of the application
 - Where a variation is considered ‘unforeseen’ (i.e. C.I.13), a brief explanation/justification is required
2. Where a variation is the implementation of wording requested by SAHPRA, reference to the associated agreement/assessment/decision should be attached to the letter of application Application form (M1.2.1)
3. Proof of payment for the Variation application (M1.2.2.1)
4. The current approved dated PI and PIL (M1.3.1.1 & M1.3.2)5. Annotated/revised proposed PI and PIL as well as the clean versions (M1.3). The annotated/revised proposed PI and PIL should be dated, initialled, paginated as page X of Y
5. Validation template for variation applications (M1.2.5)
6. For generic applications, the latest approved South African innovator PI, and if the medicine is no longer marketed in South Africa, the most recently updated SAHPRA approved generic medicine PI (M1.3.1.2)

Additional requirements for selected variation applications

7. Dear Healthcare Professional (DHCP) Letter (attached to the letter of application – M1.0.1)
8. PBRER/PSUR (M5.3.6)
9. RMP (M1.8.2)
10. Peer-reviewed literature-based studies in support of the proposed variation(s) (M5)

Clinical data [where applicable for relevant Type II variations]

11. Overview of clinical data supporting the proposed variation(s) (M2.5)
12. Synopsis of each clinical study supporting the proposed variation(s) (M2.7)
13. Clinical expert reports with data/information relevant to the proposed variation(s) of the PI (M2.5)

14. Clinical study data relevant to the proposed variation(s) of the PI (M5)
15. Studies demonstrating additional pharmacokinetic properties in special populations, if not studied previously (M5)

Reliance documentation – to be supplied for both abridged and verified reviews

16. Latest approved SmPC/ PI approved by a RRA, where applicable (M1.3.5)

Reliance documentation – to be supplied for abridged reviews of Type II variations only

17. Full assessment/evaluation reports from RRAs, if available (M1.10.5)
18. Letter of access granting SAHPRA permission to obtain unredacted reports from RRAs (attached to the letter of application – M1.0) [Not required in instances where the applicant supplies the unredacted reports of RRAs to SAHPRA directly]
19. Correspondence between the Applicant and other RRAs, concerning queries relating to safety, efficacy, risk/benefit and RMP issues, if applicable. Detailed explanation/reasons if registration/approval was refused by a Regulator with which SAHPRA aligns itself (M1.10.1)

- a. For variations submitted in eCTD format:
- b. When clinical data is required, the data submitted should be limited to the proposed variation – data in support of the initial registration alone is not required.
- c. Refer to the SAHGL-BAU-01 Reliance Guideline, which defines the evaluation pathways for variations, along with the requirements for the letter of access.

3.28 Work-sharing and extension applications

3.28.1 Work-sharing

For information on SAHPRA's reliance pathways consult the Reliance guideline.

3.28.2 Extension applications

Extensions are defined as changes to the registered product that do not fall within the scope of a variation and fundamentally alter the product, such as:

- Change in active substance (e.g., new salt, ester, complex).
- Change in strength, pharmaceutical form, or route of administration.
- Other changes specific to veterinary medicinal products to be administered to food-producing animals or change or addition of target species.

These applications fundamentally alter the terms of the initial registration and thus cannot be evaluated according to a variation's procedure. Extension applications will typically be accompanied by a new registration certificate.

In terms of procedure, extension applications will be treated as new registrations by SAHPRA.

3.29 Appendix

The following must be included as part of the submission of a variation application to SAHPRA (when relevant):

- a) Tabulated Schedule of Amendments – M1.5.2.1
- b) Medicine Register Details – M1.5.2.2
- c) Affidavit by the HCR / Applicant – M1.5.2.3

4. REFERENCES

The following related documents are referenced:

[SAHPGL-HPA-07]	General information guideline
[SAHPGL-PEM-02]	Quality and Bioequivalence Guideline
[SAHPGL-CEM-01]	Clinical guideline
[SAHPGL-CEM-03]	Guideline for Patient Information Leaflet for Human Medicines
[SAHPGL-CEM-02]	Guideline for Professional Information for Human Medicines
[SAHPGL-HPA-09]	SAHPRA eCTD Specification 3.1 and Guidance for Module 1 and Regional Information
[SAHPGL-PEM-VET-04]	General Information Guideline for Veterinary Medicines
[SAHPGL-PEM-VET-02]	Veterinary Medicines Clinical Guideline
[SAHPGL-INSP-02]	SA Guide to Good Manufacturing Practice
[SAHPGL-CEM-NS-02]	Guideline to the Scheduling of Medicines

[SAHPGL-CEM-NS-01] Application for rescheduling of a substance or medicine

[SAHPGL-FIN-01] SAHPRA Payment Guideline

[SAHGL-BAU-01] Reliance Guideline

Templates:

[GLF-HPA-03A] SAHPRA Variation Validation Template for eCTD

[GLF-PEM-02C] Template for Module 2.3_Quality Information Summary (QIS)

[GLF-PEM-02D] Template for Module 2.3_Quality Overall Summary (QOS)

[GLF-HPA-06G] Application Letter Template Post Registration Variations

5. VALIDITY

This guideline is valid for a period of 5 years from the effective date of revision and replaces the old revision (v7). It will be reviewed on this time frame or as and when required.

Appendix A

Tabulated Schedule of Amendments

Note: Landscape format is recommended for this table. A column headed “Effect of amendment” is only applicable to Module 3.2.P.1 and should NOT be included for the whole table. Only those variations /

amendments listed in the table below may be reflected in the dossier.

The amended Modules are as follows:

Modules	Comparison between Modules		Reason for amendment	Reviewer's comment
	Existing	Amended		
Example 1 is not acceptable - it will take longer to evaluate as the changes have not been identified for easy reference.				
The format of example 2 is recommended as evaluation is facilitated by the identification of the specific items.				
Example 1 3.2.P.8	Stability report: XXX	Stability report YYY replaces report XXX	Includes stability data on 2 batches stored for 36 months	
Example 2 3.2.P.8.3	Stability data on two production batches stored for 24 months (about 2 years) at 25 °C/60 % RH and for 3 months at 40 °C/75 % RH submitted.	Stability data on the same two production batches stored for 36 months at 25 °C/60 % RH included.	Extension of the shelf- life to 36 months applied for.	
	No index attached data only referred to.	Detailed index included	Administrative update to facilitate review.	

3.2.P.8.1	Shelf-life of 24 months approved.	A discussion of the results is included, and a 36-month shelf-life is inferred.	Extension of the shelf-life to 36 months applied for.	
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Note: Stability specification limits should be reflected in the stability report. Out-of-specification results should be addressed.

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Appendix B**Medicine Register Details**

The following information should be included in Module 1.5.2.2.

1. Product to which this application refers

Proprietary name of medicine	Registration/Reference number	Registered medicine (R) / Old Medicine (OM)

2. Details (if there is no change to the “Current”, indicate as such under “Proposed”, do not state N/A)

	Details on <u>Current</u> registration certificate/old medicine letter	Details on <u>Proposed</u> registration certificate/old medicine letter
Proprietary Name		
HCR/Applicant		
Name of Address		
Contact person:		

Name		
Designation		
Telephone no.		
Manufacturer		
Name and Address		
SAHPRA License No.		
SMF Reference No.		
Primary and Secondary Packer		
Name and Address		
SAHPRA License No.		
SMF Reference No.		
FPRC		
Name and Address		
SAHPRA License No.		
SMF Reference No.		
FPRR		

Name and Address		
SAHPRA License No.		
SMF Reference No.		
Formulation	If the formulation is the same, confirmation of this effect will suffice.	

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Appendix C

Affidavit by the HCR / Applicant

The following affidavit should be included in Module 1.5.2.3. The affidavit should be on company letterhead and include only those statements relevant to the application. In the case of a Transfer of the Certificate of Registration (ToHCR), this must be done by the proposed HCR.

AFFIDAVIT BY THE HCR / APPLICANT

PRODUCT NAME: { } REGISTRATION NUMBER: { }

I, {insert full name and surname} Responsible Pharmacist [as defined in Section 22C(1)(b) of Act 101 of 1965] of {insert Company name} confirm that:

1. I am in possession of the master documentation pertaining to the above-mentioned medicine.
2. This master documentation is the same as that which was in existence when the medicine was initially registered or which has been updated in accordance with amendments of the medicine registration form (MRF1/CTD) in accordance with the provisions of the regulations under the Medicines and Related Substances Control Act, 1965 (Act 101 of 1965).
3. The master documentation conforms with the Registration dossier;
4. The master documentation is properly authorised (i.e., signed and dated by at least the responsible pharmacist), *and* the quality assurance or production manager as applicable;
5. The master documentation has been supplied to the new manufacturer/packer or laboratory {state company and role} and that applicable control records have been compiled. I confirm further that I have signed these to indicate my approval that they contain all the requirements listed in the relevant master documents; namely
 - formulation and method of manufacture and packaging in-process control procedures
 - specifications pharmaceutical ingredients specifications for the final product specifications for the packaging material specifications for the label
 - specifications for the package insert

- testing procedures for the pharmaceutical ingredients testing procedures for the final product
 - testing procedures for the packaging materials.
6. I confirm that a technical agreement and/or signed contract(s) exist(s) with all third-party manufacturer(s)/packer(s)/laboratory(ies) involved in manufacturing of this product.
7. For an alternative/additional manufacturer:
- I confirm that the manufacturing procedure (including equipment) is identical to the manufacturing procedure currently used or
 - I confirm that the manufacturing procedure (including equipment) differs, but falls within the <Type IA><Type IA_{IN}> and <Type IB> amendments or
 - I confirm that the manufacturing procedure (or equipment) is different from the manufacturing procedure (or equipment) currently on file outside of the <Type IA><Type IA_{IN}> and <Type IB> amendments and that comparative data (efficacy), stability data or protocol (as applicable), and a validation protocol for the first three production batches, are submitted.
8. <I confirm that the PI will be updated to reflect the new HCR details and will submit the amended PI with the first update of the dossier after authorisation of this amendment. (for ToHCRs only)>
9. <I confirm that the Registration dossier will be fully updated to the current statutory format and current scientific standards within 12 months of transfer of the certificate of registration, or approval of additional, or change of manufacturer.>

OR

<I confirm that the Registration dossier will be fully updated to the current statutory format and current scientific standards by (stipulate date) in accordance with the programme as approved by the Inspectorate.>

Signed on {DD/MM/YYYY}:
